

# Appendices

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## Appendix A: Abbreviation Glossary

|                                                                   |                                                                              |
|-------------------------------------------------------------------|------------------------------------------------------------------------------|
| <b>ADA:</b> Americans with Disabilities Act                       | <b>HQS:</b> Housing Quality Standards                                        |
| <b>AED:</b> Automated External Defibrillator                      | <b>HR:</b> Human Resources                                                   |
| <b>AI:</b> Artificial Intelligence                                | <b>HRSA:</b> Health Resources and Services Administration                    |
| <b>CDC:</b> Centers for Disease Control                           | <b>HUD:</b> Department of Housing and Urban Development                      |
| <b>CDL:</b> Commercial Driver's License                           | <b>IDA:</b> Individual Development Account                                   |
| <b>CHI:</b> Community Health Integration                          | <b>IRS:</b> Internal Revenue Service                                         |
| <b>CHW:</b> Community Health Worker                               | <b>LIHTC:</b> Low-Income Housing Tax Credit                                  |
| <b>CRM:</b> Customer Relationship Management                      | <b>LLC:</b> Limited Liability Company                                        |
| <b>CTS:</b> Certified Transport Specialist                        | <b>MTM:</b> Medically Tailored Meals                                         |
| <b>DOT:</b> Department of Transportation                          | <b>NEMT:</b> Non-Emergency Medical Transportation                            |
| <b>EBT:</b> Electronic Benefits Transfer                          | <b>NEMTAC:</b> Non-Emergency Medical Transportation Accreditation Commission |
| <b>EIN:</b> Employer Identification Number                        | <b>SDOH:</b> Social Determinants of Health                                   |
| <b>GPS:</b> Global Positioning System                             | <b>UBIT:</b> Unrelated Business Income Tax                                   |
| <b>HCBS:</b> Home and Community-Based Services                    | <b>WAV:</b> Wheelchair Accessible Vehicle                                    |
| <b>HCV:</b> Housing Choice Voucher                                |                                                                              |
| <b>HIPAA:</b> Health Insurance Portability and Accountability Act |                                                                              |

## Appendix B: Equipment Vendor Contacts

### Vehicle Dealers and Conversion Companies

- BraunAbility
- MobilityWorks
- Vantage Mobility

### Technology Vendors

- Verizon Connect
- Samsara
- Salesforce

### Medical Equipment Suppliers

- Stryker

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- Ferno
- Zoll

Training Provider Contacts

CHW Training Programs

- National Association of Community Health Workers (NACHW)
- State Universities
- Local Community Colleges

NEMT Training Organizations

- NEMTAC
- CTAA
- XTransit Solutions

Appendix C: Sample Budget Worksheet

Startup Costs Summary (Revised)

| Category                           | Low Estimate | High Estimate |
|------------------------------------|--------------|---------------|
| Legal and Formation                | \$8,600      | \$17,500      |
| Technology Systems                 | \$15,000     | \$25,000      |
| Office Setup                       | \$10,000     | \$20,000      |
| Vehicle Fleet (Revised)            | \$120,000    | \$188,000     |
| Equipment                          | \$50,000     | \$100,000     |
| Training                           | \$18,500     | \$68,500      |
| Working Capital                    | \$50,000     | \$100,000     |
| Total Initial Investment (Revised) | \$272,100    | \$519,000     |

Monthly Operating Costs (Revised)

| Category                 | Low Estimate | High Estimate |
|--------------------------|--------------|---------------|
| Salaries and Benefits    | \$20,000     | \$35,000      |
| Vehicle Operations       | \$8,000      | \$15,000      |
| Insurance                | \$7,500      | \$15,000      |
| Technology               | \$2,500      | \$5,000       |
| Administrative (Revised) | \$4,500      | \$8,166       |
| Total Monthly (Revised)  | \$42,500     | \$78,166      |

Appendix D: Key Performance Indicators (KPIs)

Operational KPIs

- **Client Satisfaction Score:** Target 90%+
- **On-Time Performance:** Target 95%+
- **Vehicle Utilization Rate:** Target 75%+
- **Driver Retention Rate:** Target 85%+
- **Safety Incidents:** Target <1 per 10,000 miles

Financial KPIs

- **Revenue Growth Rate:** Target 25%+ annually
- **Operating Margin:** Target 15%+ by Year 3
- **Cost per Client Served:** Target decrease 10% annually
- **Revenue per Vehicle:** Target \$50,000+ annually
- **Accounts Receivable Days:** Target <45 days

Impact KPIs

- **Number of Clients Served:** Target 500+ in Year 1
  - **Health Outcomes Improved:** Target 80% of clients
  - **Housing Placements:** Target 50+ annually
  - **Meals Delivered:** Target 10,000+ annually
  - **Community Partnerships:** Target 25+ organizations
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## Appendix E: Regulatory Compliance Checklist

### Federal Requirements

- IRS 501(c)(3) application filed
- Federal EIN obtained
- Medicare provider enrollment completed
- HIPAA compliance program established
- DOT compliance for commercial vehicles

### State Requirements (Alabama)

- Alabama nonprofit registration
- Business licenses obtained
- Insurance producer licenses
- Medicaid provider enrollment
- Vehicle registrations and inspections

### Local Requirements

- Business permits and licenses
- Zoning compliance for facilities
- Health department permits for food service
- Fire department inspections
- Building permits for modifications

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The Porch Light Blueprint

*This comprehensive implementation plan provides detailed guidance for establishing and operating the Porch Light Foundation hybrid model. Regular updates should be made to reflect changing regulations, market conditions, and operational experience.*



# The Porch Light Blueprint

A Comprehensive, Actionable Plan for  
Serving Tuscaloosa's Underserved Communities

The Porch Light Blueprint

by Quincy Ruffin

2025

The Porch Light Blueprint

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# Executive Summary

This report presents a comprehensive strategic blueprint for "Porch Light," a non-profit organization designed to address the critical, interconnected health and social needs of Tuscaloosa, Alabama, and its surrounding counties. An analysis of the region's public health data and community assessments reveals significant, persistent challenges, including high rates of chronic disease, formidable barriers to accessing healthcare, widespread food and housing insecurity, and specific gaps in transportation, childcare, and workforce development that isolate vulnerable populations. [1, 2, 3, 4, 5] Porch Light is conceived to address these challenges directly through a deeply integrated, multi-pillar service model designed to support individuals and families across the lifespan.

## The core pillars of the Porch Light model include:

- **Foundational Services:** Addressing core insecurities through Porch Light Rides (a facilitated Non-Emergency Medical Transportation service), Porch Light Plates (a Medically Tailored Meal program), and Porch Light Homes (a supportive housing initiative).
- **Family & Lifespan Support:** Providing holistic care through a Community-Based Doula Program for maternal health, a Family Resource Center offering affordable childcare and parenting support, and Comprehensive Senior Services to help older adults age in place with dignity.
- **Economic Empowerment:** Building long-term stability through integrated Workforce Development services, a Financial Empowerment Center offering one-on-one financial counseling, and an Individual Development Account (IDA) matched savings program.

The strategic foundation of this plan is a diversified and resilient funding model that braids earned revenue from federal and state health programs with traditional philanthropic support. Porch Light will leverage Alabama's specific Medicaid reimbursement structures for NEMT, Home and Community-Based Services (HCBS) waivers, and childcare subsidies. [6, 7, 8]

Critically, the plan incorporates the groundbreaking 2024 changes to Medicare, which now allow for direct reimbursement for services that address the Social Determinants of Health (SDOH). [9, 10] This allows Porch Light to operate as a sustainable social enterprise, where its central intake and care coordination hub, Porch Light Connect, generates revenue that supports and expands its direct service programs.

## Key Innovation Points:

- **Circular Impact Model:** For-profit subsidiaries generate revenue funding nonprofit activities
- **AI-Driven Operations:** Intelligent Algorithm Management serves as centralized hub with predictive analytics
- **Insurance Integration:** Algoinsu provides affordable coverage through Porch Light subsidies

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- **Comprehensive Tracking:** Advanced GPS and client management systems ensure operational efficiency

#### **Financial Projections Summary:**

- **Year 1:** \$200,000–\$375,000 total revenue
- **Year 2:** \$550,000–\$1,000,000 total revenue
- **Year 3:** \$1,150,000–\$1,850,000 total revenue
- **Total Initial Investment Required (Revised):** \$272,100–\$519,000

This document provides an actionable roadmap for program implementation, provider certification, staffing, and financial sustainability, positioning Porch Light to become an indispensable part of the West Alabama community health ecosystem.

# Part I: The Opportunity Landscape: Needs and Systems in West Alabama

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## Section 1: A Community in Need: The Health and Social Landscape of West Alabama

To build effective programs, Porch Light's mission must be grounded in a data-driven understanding of the specific challenges facing the residents of Tuscaloosa and its surrounding counties. A review of local health assessments and demographic data reveals a community grappling with significant health disparities and systemic barriers to well-being.

### Chronic Disease Burden and Access to Care

The DCH Health System's Community Health Needs Assessment (CHNA) consistently identifies chronic disease management and access to healthcare as top priorities for the region. [1, 11] The seven-county service area ranks poorly in Alabama for health outcomes, driven by high rates of obesity, diabetes, and hypertension. Alabama has one of the highest rates of hypertension in the country, and in some local counties, the diabetes rate has reached a staggering 23%. [2, 12] These conditions are exacerbated by a severe shortage of providers; Tuscaloosa County is a federally designated Health Professional Shortage Area (HPSA) for primary, dental, and mental health care. This forces many rural and low-income residents to either forgo care or use hospital emergency departments for primary care needs, a costly and inefficient solution. [2]

### Pervasive Social Determinant of Health (SDOH) Barriers

#### Transportation

Transportation is a major cross-cutting barrier identified repeatedly as a hindrance to health and wellness in West Alabama. [2] Research from the University of Alabama highlights a severe geographic mismatch between residential areas of vulnerable populations (like the Latino/a/x community) and the location of primary care providers, who are clustered in the Tuscaloosa city center. [3, 13, 14] This is compounded by a lack of public transit options connecting these communities to essential services. [3, 14]

#### Food Insecurity

The region suffers from high rates of food insecurity, with 17% of adults and 23% of children in Alabama lacking regular access to nutritious food. In Tuscaloosa County, the child food insecurity rate is 19.8%, affecting an estimated 8,500 children and creating an annual food budget shortfall of over \$19.2 million. This lack of healthy food is directly linked to poor nutrition and a greater risk for chronic disease.

#### Housing Instability

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The lack of safe, affordable housing is a public health crisis. The West Alabama Coalition for the Homeless consistently finds a significant gap between available resources and the number of individuals and families

needing assistance.

## Childcare Crisis

Alabama has a statewide childcare supply gap of 40%, meaning there are only enough licensed slots for 60% of children under age 6 with working parents. In Tuscaloosa County, this shortage has an estimated annual economic impact of up to \$143.5 million. The cost is also a major barrier, with the average annual price for infant care (\$7,871) consuming nearly 10% of a median family's income, well above the federal affordability threshold of 7%.

## Needs of Specific Vulnerable Populations

### Children and Families

The 2022 CHNA from Children's of Alabama identifies mental health, access to care, and child safety as the top concerns for all age groups. [15, 16] For children 0-5, key issues include parental substance use and a lack of safe, affordable childcare. [15, 16] For school-aged children and teens, mental health becomes the overwhelming priority, with high rates of depression, anxiety, and trauma, and a lack of support services for issues like substance abuse. [15, 16]

### Pregnant Women

Pregnant women, particularly those from minority and low-income backgrounds, face significant disparities. In Alabama, the infant mortality rate for Black infants has been nearly three times that of white infants. These women often require material assistance, stable housing, mental health support for conditions like postpartum depression, and reliable transportation to frequent prenatal appointments. [17, 18, 19, 20]

### The Elderly

The West Alabama Area Agency on Aging identifies critical needs for the region's seniors, including access to nutritious meals, transportation, in-home assistance with daily activities, and support services for family caregivers, such as respite care. [21, 22, 23]

## Section 2: The Funding Framework: Navigating Public and Private Revenue Streams

Porch Light's sustainability hinges on its ability to strategically leverage a complex but powerful blend of government healthcare reimbursements, public grants, and private philanthropy. This braided funding model is the financial engine that will support the organization's integrated services.

## Alabama Medicaid and HCBS Waivers

### Non-Emergency Transportation (NET)

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Alabama's Medicaid NET program is a cornerstone of the Porch Light Rides initiative. Unlike systems where providers bill the state directly, Alabama's program often reimburses the Medicaid recipient via an Electronic

Benefit Transfer (EBT) card after a trip is completed. [6, 24, 25] The recipient must call in advance to get a 7-digit "ticket number" to authorize the trip. [26, 27] While this presents collection challenges, it also creates an opportunity for Porch Light to act as an essential navigator for clients. Crucially, state procedures allow for direct payment to the transportation provider for recurring services like dialysis and cancer treatment, creating a vital pathway for stable, bankable revenue.

## Home and Community-Based Services (HCBS) Waivers

HCBS waivers are a crucial Medicaid tool that allows states to fund services that help people live in their communities instead of institutions. [7, 28, 29] Several Alabama waivers, including the Elderly and Disabled (E&D) Waiver and the Alabama Community Transition (ACT) Waiver, explicitly cover services that align with Porch Light's mission, such as home-delivered meals, personal care, homemaker services, and respite care. [30, 31, 32] By becoming a certified provider for these waivers, Porch Light can receive direct, fee-for-service reimbursement for these services.

## The Medicare SDOH Revolution (Effective January 1, 2024)

A pivotal development is the Centers for Medicare & Medicaid Services (CMS) decision to begin reimbursing for services that address Health-Related Social Needs (HRSNs). This policy shift allows organizations like Porch Light to generate revenue from care coordination activities that were previously unfunded. The key mechanisms are new Healthcare Common Procedure Coding System (HCPCS) codes that can be billed by a clinical partner when services are provided by "auxiliary personnel," such as Porch Light's community health workers. [9, 10]

- **HCPCS Code G0136:** Pays for administering a standardized SDOH risk assessment. [33, 34]
- **HCPCS Codes G0019 & G0022:** Pay for Community Health Integration (CHI) services—the time spent helping a patient address identified SDOH needs. [9, 10]

This new framework is a game-changer, creating a financially sustainable model for the Porch Light Connect hub and reinforcing the entire ecosystem of care.

## State and Federal Grant Opportunities

Beyond earned revenue, Porch Light will actively pursue grant funding for startup capital, pilot programs, and service expansion. Key sources include:

### Federal Housing and Community Development

The U.S. Department of Housing and Urban Development (HUD) offers numerous programs like the Community Development Block Grant (CDBG) and HOME Investment Partnerships Program, administered locally by the City of Tuscaloosa, which are vital for the Porch Light Homes initiative. [35, 36, 37]

### Maternal and Child Health

The federal Transforming Maternal Health (TMaH) Model, which Alabama is a participant in, provides a 10-year opportunity for community-based organizations to partner with the state to improve maternal health outcomes. [38, 39, 40] Additionally, grants from the Children's Bureau (e.g., PSSF, CBCAP) and the 21st Century Community Learning Centers program can support the Family Resource Center.

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### Local and State Foundations

Organizations like the Community Foundation of West Alabama, the Alabama Power Foundation, and The Daniel Foundation of Alabama offer grants that align with Porch Light's mission in health, social welfare, and youth development. [41, 42, 43]

## Part II: Foundational Pillars: Addressing Core Insecurities

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### Section 3: "Porch Light Rides" - A Facilitated Non-Emergency Medical Transportation (NEMT) Service

The "Porch Light Rides" program is designed to be more than a simple taxi service; it is a facilitated transportation solution tailored to the unique realities of Alabama's Medicaid NET system and the needs of its target clients. The program will provide reliable, safe, and friendly non-emergency medical transportation, with a primary focus on Medicaid-eligible elderly individuals and pregnant women in Tuscaloosa County and its underserved surrounding rural areas. [2, 44]

#### Operational Blueprint and Pathways to Direct Reimbursement

The core of the service model is active facilitation. Staff will not only provide the physical ride but will also provide "white-glove" assistance to clients, helping them navigate the state's NET call-in and reimbursement process, a significant barrier for many. [6, 26] However, to build a bankable revenue stream, the strategic priority is to secure direct payment from Medicaid, bypassing the recipient-reimbursement model whenever possible.

#### Targeting the Direct Payment Exception

The most immediate path to direct reimbursement is by focusing on clients with recurring appointments. Alabama Medicaid procedures state that for recipients attending dialysis, cancer treatments, or other regular appointments, the treatment facility verifies attendance, and "If a transporter provides transportation, payment is made directly to the transporter." Porch Light will proactively establish partnerships with dialysis clinics, oncology centers, and other high-frequency treatment facilities to become their preferred transportation provider, creating a stable, direct-billed revenue stream.

#### Petitioning for Redirected Payments

For other clients, Porch Light will leverage the administrative rule allowing Medicaid to "redirect the reimbursement on behalf of the recipient to an address determined to be more appropriate." [45] The organization will make a formal case to the agency that for vulnerable clients (e.g., those with cognitive impairments, frequent hospital discharges, or documented inability to manage funds), Porch Light is the "more appropriate" entity to receive payment, framing this as a more efficient and reliable method to ensure access to care. [46]

#### Standard EBT Facilitation

For all other trips, Porch Light will manage the recipient-reimbursement process. A trained staff member will assist the client in calling the Medicaid NET hotline (1-800-362-1504) to secure the mandatory 7-digit ticket number. [27] After the ride, Porch Light will assist the client in using their EBT card to pay for the service once the state reimbursement is processed, which can take 60-90 days. [27]

## Compliance, Documentation, and Staffing

Strict adherence to Medicaid regulations is paramount.

### Documentation

For every trip, Porch Light must meticulously document the beneficiary's name and Medicaid ID, pickup and drop-off locations and times, and the total loaded mileage traveled with the beneficiary in the vehicle. [47]

### Compliance

Drivers must be rigorously trained on what constitutes fraud, such as providing rides to non-medical locations like the grocery store, which is strictly prohibited. [23]

### Vehicles and Drivers

The program will require a fleet of reliable and fully insured vehicles, including at least one wheelchair-accessible van. [47, 48, 49] All drivers must undergo and pass criminal background checks, possess a valid driver's license, and receive comprehensive training on passenger assistance, safety protocols, HIPAA, and cultural sensitivity. [47, 50, 48, 49]

## Section 4: "Porch Light Plates" - A Medically Tailored Meal and Nutrition Service

The "Porch Light Plates" program will operate on the "Food is Medicine" principle, positioning itself as a clinical support service that provides Medically Tailored Meals (MTMs) to individuals with specific health conditions. This approach aligns directly with the standards of Alabama's HCBS waivers and the successful models of national organizations like God's Love We Deliver and Mom's Meals. [51, 52, 53]

### Service Model and Menu Development

The program will go beyond a standard food pantry model. In partnership with a staff or consultant Registered Dietitian (RD), Porch Light will develop and prepare meals that meet the specific nutritional requirements of clients with chronic conditions highly prevalent in Tuscaloosa, such as diabetes, heart disease, and kidney disease. [7, 54] Menus will include:

- **Diabetes-Friendly:** Controlled for carbohydrates and sugar to help manage blood glucose levels. [55]
- **Heart-Friendly:** Lower in sodium and saturated fat to support cardiovascular health. [54]
- **Renal-Friendly:** Designed to meet the complex nutritional needs of patients with kidney disease. [55]
- **Texture-Modified:** Including pureed or soft-food options for individuals with difficulty chewing or swallowing (dysphagia). [54]

### Provider Certification and Reimbursement

Becoming a reimbursed provider involves a dual-approval process:



1. **Programmatic Approval:** Porch Light must first request and submit a detailed Meal Delivery Services Provider Application Packet to the appropriate state operating agency (e.g., ADSS for the E&D waiver, ADMH for the ID waiver). [7] This packet must include a comprehensive service plan, sample menus with nutritional analysis, and robust food safety and delivery protocols. [7]
2. **Medicaid Enrollment:** Following approval from the operating agency, Porch Light must enroll with the Alabama Medicaid Agency as a Meal Delivery Services provider. [7, 56] This step involves adding the applicable billing codes to the organization's Medicaid provider profile, which allows for claim submission and reimbursement. [7, 56]

Reimbursement for home-delivered meals under HCBS waivers is based on an "actual cost" model. [57, 58] The state does not have a fixed fee schedule; instead, the provider's rate is determined based on audited past performance and renegotiated contracts. [57, 58]

## The Community Nutrition Hub

To address broader community needs, Porch Light Plates will also function as a Community Nutrition Hub. This expanded model will include:

- **Client-Choice Pantry:** Modeled after local pantries like Table of Grace, this allows clients to select their own food with dignity.
- **Benefits Enrollment:** Staff will provide one-on-one assistance to help families enroll in SNAP and WIC.
- **Nutrition Education:** The hub will host classes and demonstrations on healthy cooking and diet management, in partnership with local experts. [2, 59]
- **Referral Pipeline:** The hub will serve as a natural referral source for the MTM program, identifying pantry clients who may be eligible for waiver-reimbursed meals. [21]

## Section 5: "Porch Light Homes" - A Supportive Housing Partnership

Recognizing that stable housing is a foundational social determinant of health, the "Porch Light Homes" initiative will address housing insecurity through a strategic public-private partnership. This pillar will connect clients identified through the "Connect" hub with safe, affordable housing, leveraging the expertise of a for-profit partner like 20/59 Ventures to manage properties while Porch Light focuses on client support and placement.

### A Public-Private Partnership Model

This initiative is built on a joint venture between Porch Light and a for-profit entity specializing in real estate. This model leverages the unique strengths of each partner: [60, 61, 62]

- **Porch Light's Role (Non-Profit):** As the client-facing entity, Porch Light will be responsible for identifying and screening eligible individuals and families, providing comprehensive case management, facilitating the placement process, and offering ongoing supportive services to ensure housing stability.
- **20/59 Ventures' Role (For-Profit):** As the housing expert, 20/59 Ventures will manage property acquisition, renovations, day-to-day property management, and ensure all properties meet the rigorous standards required by housing assistance programs.

## Operational Framework: The Section 8 Housing Choice Voucher (HCV) Program

The primary mechanism for providing affordable housing will be the federal Section 8 Housing Choice Voucher (HCV) program, administered locally by the Tuscaloosa Housing Authority (THA). [63, 64] The HCV program allows low-income families to lease privately-owned rental housing. [65]

The operational steps for the partnership are as follows:

1. **Client Identification:** Porch Light's CHWs will use SDOH screening tools to identify clients experiencing homelessness or housing instability, documenting these needs with ICD-10 Z codes (e.g., Z59.0 for homelessness). [66]
2. **Voucher Application:** Porch Light will assist eligible clients in applying for an HCV from the THA.
3. **Property Placement:** Once a client has a voucher, Porch Light will connect them with an available, appropriate unit managed by 20/59 Ventures.
4. **Landlord & PHA Approval:** As the landlord, 20/59 Ventures will work with the THA to complete the necessary steps for approval: [67, 68]
  - **Request for Tenancy Approval:** Submitting the required HUD form. [68, 69]
  - **Housing Quality Standards (HQS) Inspection:** The THA must inspect the unit to ensure it meets federal health and safety standards. [65, 69, 70] 20/59 Ventures is responsible for all maintenance to meet these standards. [65]
  - **Rent Reasonableness:** The THA will determine if the proposed rent is reasonable compared to similar unassisted units in the area. [65, 69, 71]
  - **HAP Contract:** Once approved, 20/59 Ventures and the THA will sign a Housing Assistance Payment (HAP) contract, which governs the subsidy payments. [72, 73]

## Legal and Financial Structure

To formalize the collaboration, Porch Light and 20/59 Ventures will form a joint venture, likely structured as a Limited Liability Company (LLC) or Limited Partnership (LP). [74, 75, 23, 76] This legal structure is common for affordable housing projects and serves to clearly define roles and responsibilities while protecting the non-profit's charitable assets. [74, 75, 23, 76] This partnership model unlocks powerful funding mechanisms crucial for acquiring and developing affordable housing, most notably the Low-Income Housing Tax Credit (LIHTC), which provides significant federal incentives for affordable rental housing production. [75, 77, 53, 78, 79]

## Part III: A Holistic Framework for Health and Family Well-Being

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### Section 6: "Porch Light Connect" - A Community Health Integration Hub

"Porch Light Connect" is the innovative nerve center of the organization. It functions as a client intake and care coordination hub that leverages the new Medicare SDOH reimbursement codes to create a sustainable revenue stream that feeds clients into the Rides, Plates, and Homes programs. This pillar will eventually evolve into a full-fledged Community Care Hub (CCH), a central entity that organizes and contracts for social care services across the region. [80, 81, 82]

#### Service Model: The Community Health Worker (CHW) Initiative

The model is built on partnership. Porch Light will establish formal contracts with local healthcare providers (physicians, clinics, FQHCs). Porch Light's trained, non-clinical staff—acting as Community Health Workers (CHWs)—will provide billable services "incident to" the services of the partner clinician. [9, 10] This means the clinical partner bills Medicare for the work performed by Porch Light's staff.

#### Operational Workflow and Billing

The process creates a financially reinforcing loop:

1. **Referral & Consent:** A partner clinic identifies a Medicare patient with unmet social needs and obtains consent for CHI services. [9, 11]
2. **SDOH Risk Assessment (Billable Service):** A Porch Light CHW administers a standardized assessment tool (e.g., PRAPARE). [33, 34, 83, 84] The partner clinic then bills Medicare for this assessment using HCPCS code G0136 (approx. \$18.97). [85, 86]
3. **Documentation (Critical Step):** The CHW documents the identified needs in the patient's medical record using the appropriate ICD-10-CM Z codes (e.g., Z59.41 for food insecurity, Z59.82 for transportation insecurity). [87, 88] This documentation is the justification for all subsequent billing. [87, 88]
4. **Community Health Integration (Billable Service):** The CHW then spends time providing CHI services—navigating the system and connecting the patient to resources. The partner clinic bills Medicare for the CHW's time using HCPCS code G0019 for the first 60 minutes per month and G0022 for each additional 30 minutes. [10] Reimbursement is approximately \$80 for the first hour and \$50 for each additional 30 minutes for office-based practitioners. [89]
5. **Integrated Internal Referral:** The "resource" the CHW connects the patient to will frequently be Porch Light's own programs, creating a seamless client journey and maximizing the organization's impact.

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**Table 1: Medicare SDOH Reimbursement Codes: A Guide for Porch Light**

| HCPSC Code  | Service Description                  | Billing Frequency             | Approximate Reimbursement |
|-------------|--------------------------------------|-------------------------------|---------------------------|
| G0136       | SDOH Risk Assessment                 | Once per beneficiary per year | \$18.97                   |
| G0019       | CHI Services (first 60 minutes)      | Once per month                | ~\$80                     |
| G0022       | CHI Services (additional 30 minutes) | As needed per month           | ~\$50                     |
| G0023/G0024 | Principal Illness Navigation         | Specialized conditions        | Varies                    |

## Section 7: "Porch Light Families" - A Lifespan Approach to Family Support

This pillar houses a suite of programs designed to support families at critical life stages, addressing needs identified as acute in West Alabama.

### 7.1 Program Initiative: Community-Based Doula Program

To address the significant maternal and infant health disparities in the region, Porch Light will establish a community-based doula program. Doulas provide non-clinical emotional, physical, and informational support to mothers before, during, and after childbirth, an evidence-based practice shown to improve birth outcomes, reduce C-sections, and increase breastfeeding rates.

#### Model

The program will be a community-based model, recruiting and training doulas who reflect the communities they serve to provide culturally congruent care. [90] Services will include prenatal and postpartum home visits, continuous labor support, and referrals to address social needs like housing and transportation. [90, 91]

#### Training

Doulas will be trained using an evidence-based, comprehensive curriculum (such as those offered by DONA International or HealthConnect One) that includes core competencies in communication, advocacy, trauma-informed care, and lactation support.

#### Funding and Reimbursement

While Alabama does not yet have a statewide Medicaid reimbursement plan for doula services, there are multiple funding pathways. [92, 93] The program will pursue a blended funding model including:

- **Grants:** Seek funding from the Children's Trust Fund of Alabama, the TMaH model, and private foundations.

- **Partnerships with MCOs and Hospitals:** Contract with Managed Care Organizations (MCOs) and hospitals to provide doula services as an expanded or value-based benefit, demonstrating ROI through reduced C-section rates and improved outcomes. [24, 94, 95, 40]
- **Private Pay:** Offer services on a sliding scale for clients who can afford to pay. [93]

## 7.2 Program Initiative: Family Resource Center & Childcare

To address the dual crises of childcare affordability and accessibility, Porch Light will establish an integrated Family Resource Center.

### Licensed Childcare Facility

The cornerstone is a high-quality childcare facility licensed by the Alabama Department of Human Resources (DHR), serving children from infants to school-age. [78, 96, 97]

### Child Care Subsidy Program

The center will enroll with DHR to accept payments from the Child Care Subsidy Program, making care affordable for low-income working families. [8, 79]

### Wrap-Around Services

The center will offer evidence-based parenting education classes, after-school and summer enrichment programs, and support groups, funded through a mix of grants and partnerships.

### Innovative Models

Porch Light will explore public-private partnerships like Michigan's "Tri-Share" model, where the cost of childcare is split between the employer, the employee, and the state, framing it as a workforce development tool for local businesses.

## 7.3 Program Initiative: Comprehensive Services for Seniors and Caregivers

To meet the needs of the region's aging population, Porch Light will become an enrolled provider for Alabama's E&D and ACT Medicaid waivers, offering a bundled suite of reimbursable services. [30, 98]

### Bundled Services

The program will provide a coordinated package of care based on a client's Plan of Care, including: [99, 57]

- **Personal Care:** Assistance with activities of daily living (ADLs) like bathing and dressing. [21, 22, 57]
- **Homemaker Services:** Help with tasks like light housekeeping and meal prep. [21, 57]
- **Companion Services:** Supervision and social engagement to prevent isolation. [21, 57]
- **Respite Care:** Temporary relief for family caregivers. [21, 22, 23, 57]

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### Case Management Coordination

Porch Light will work in close coordination with the case managers from the local Area Agency on Aging to deliver authorized services and ensure seamless care for seniors. [56, 99]

# Part IV: Building a Sustainable and Empowering Organization

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## Section 8: Pathways to Economic Empowerment

A core tenet of Porch Light's philosophy is that providing social services must be linked to creating pathways to economic opportunity. The organization will integrate workforce and financial empowerment services into its core operations, adopting a "no wrong door" approach to helping clients achieve long-term stability.

### 8.1 Program Initiative: Integrated Workforce Development

Porch Light will bridge the gap between its clients and the region's existing workforce development ecosystem, which includes partners like West Alabama Works!, Shelton State Community College, and the Tuscaloosa Career Center. [100, 101, 13]

#### Formal Partnerships

Establish co-location agreements where career coaches from partner agencies hold office hours at Porch Light facilities. [102, 103, 104]

#### Professional Clothing Closet

Create a "Business Boutique" to provide clients with free, professional attire for job interviews, modeled after successful programs like Bottomless Closet.

#### Digital Literacy and Job Application Access

Establish a computer lab with free internet access and digital navigator support to help clients build digital skills, search for jobs, and complete online applications. [100, 105, 106]

### 8.2 Program Initiative: Financial Empowerment Center (FEC)

Porch Light will adopt the nationally recognized Financial Empowerment Center (FEC) model to provide free, one-on-one financial counseling to community members.

#### Services

Professionally trained counselors will help clients with budgeting, debt reduction, credit building, and connecting to safe banking products.

#### Integration

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FEC services will be integrated with other Porch Light programs, such as housing and workforce development, to provide holistic support.

### **Individual Development Accounts (IDAs)**

The FEC will partner with the United Way of West Alabama to enroll eligible clients in the IDA program. This federal program provides matched savings accounts to help low-income families build assets for purchasing a first home, paying for post-secondary education, or starting a business.

## **Section 9: Corporate Structure Strategy**

The Porch Light Foundation utilizes an innovative hybrid nonprofit-for-profit model addressing community health and social needs through integrated services.

### **Recommended Entity Structure**

#### **Intelligent Algorithm Management (Hub Entity)**

An LLC or Corporation serving as the centralized operations and AI-driven analytics hub, owning and controlling subsidiaries. Formation cost: \$200–\$500.

#### **Porch Light Foundation (Nonprofit)**

A 501(c)(3) nonprofit organization focused on mission-driven services and serving as the grant recipient. Formation cost: \$600–\$1,200.

#### **For-Profit Subsidiaries (Corporations)**

Include Transportation, Properties, Logistics, Foods, and Tech companies.

#### **Algoinsu Insurance Brokerage**

A corporation focused on AI-driven insurance sales. Formation cost: \$300–\$800. Algoinsu's operating expenses and launch costs will be managed independently.

## **Section 10: Equipment and Technology Needs**

### **Core Technology Infrastructure**

#### **AI-Driven Management System**

Utilizes CRM platforms like Salesforce or HubSpot. Annual cost: \$9,000–\$36,000.

#### **GPS Tracking and Fleet Management**



Systems like Verizon Connect or Samsara. Annual cost: \$3,100–\$4,500.

Commercial Kitchen Setup

Total setup costs: \$31,000–\$54,000.

Office Equipment and Furniture

Basic office setup: \$5,000–\$10,000 for furniture and \$800–\$1,500 each for computers/laptops.

Section 11: Vehicle Fleet Requirements and Costs

The revised strategy for Year 1 involves scaling down the initial fleet acquisition to 3-4 vehicles to reduce startup costs.

Original vs. Revised Fleet Plan

Original Fleet Plan

Required 3-5 WAVs, 5-8 Standard Medical Transport Vehicles, and 2-3 Meal Delivery Vehicles, totaling 10-16 vehicles initially. Original Initial Fleet Cost: \$426,000–\$985,000.

Revised Year 1 Fleet (3-4 Vehicles)

Revised Initial Fleet Cost: \$120,000–\$188,000

Vehicle Cost Breakdown

| Vehicle Type                  | Cost Range               | Purpose                         |
|-------------------------------|--------------------------|---------------------------------|
| New Converted Minivans (WAVs) | \$65,000 - \$98,000 each | Wheelchair accessible transport |
| Used Minivans (Standard)      | \$15,000 - \$25,000 each | Standard medical transport      |
| Cargo Vans (Meal Delivery)    | \$25,000 - \$40,000 each | Food delivery service           |

Maintenance and Compliance

- **Annual Operating Costs:** \$75,000–\$150,000
- **Insurance Costs:** \$60,000–\$120,000 annually

Section 12: Training and Certification Requirements

The total initial training investment is estimated at \$18,475–\$68,535.

## Training Programs

### Community Health Worker (CHW) Training

Includes a 160-hour competency-based training program.

### NEMT Driver Training and Certification

Includes NEMTAC Certification.

### Kitchen and Food Service Training

Includes ServSafe Certification.

### Administrative and Management Training

Includes insurance license training.

## Section 13: Implementation Timeline with Costs

### Year 1: Foundation Building (Months 1-12)

#### Phase 1: Legal Structure Formation (Months 1-3)

Cost: \$8,600–\$17,500

#### Phase 2: Infrastructure Development (Months 4-6)

Cost: \$58,000–\$100,000

#### Phase 3: Program Launch (Months 7-9)

- Vehicle Acquisition (Revised): \$120,000–\$188,000
- Equipment Purchase: \$20,000–\$40,000
- Insurance Premiums: \$30,000–\$60,000
- Marketing and Outreach: \$5,000–\$10,000
- **Total Phase 3 (Revised): \$175,000–\$298,000**

#### Phase 4: Operations Stabilization (Months 10-12)

Cost: \$100,000–\$185,000

**Year 1 Total Investment (Revised): \$341,600–\$600,500**

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### Year 2: Subsidiary Launch (Months 13-24)

- Properties Company Launch: \$50,800–\$202,300
- Logistics Company Launch: \$85,300–\$175,800
- **Year 2 Total Investment (Revised): \$136,100–\$378,100**

## Year 3: Scale and Expansion (Months 25-36)

- Geographic Expansion: \$140,000–\$280,000
- Additional Subsidiaries (Foods and Tech): \$80,000–\$175,000
- **Total Year 3 Investment: \$220,000–\$455,000**

**Three-Year Total Investment (Revised): \$697,700–\$1,434,600**

## Section 14: Risk Management and Compliance

### Transportation Industry Risks

High-risk exposure areas include vehicle accidents, medical emergencies, driver misconduct, and regulatory compliance.

### Risk Mitigation Strategies

The plan emphasizes a robust corporate structure, comprehensive insurance, rigorous driver training, and safety protocols.

### Insurance Coverage

- **Commercial Auto Insurance:** \$8,000–\$15,000 per vehicle annually
- **General Liability Insurance:** \$15,000–\$30,000 annually
- **Umbrella Coverage:** \$5,000,000–\$10,000,000

### Regulatory Compliance

- **Healthcare Regulations:** HIPAA, Medicare/Medicaid
- **Transportation Regulations:** DOT, ADA
- **Insurance Regulations:** State licensing requirements

## Section 15: Financial Projections

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## Year 1 Financial Projections

### Year 1 Revenue and Expenses

| Category                     | Low Estimate | High Estimate |
|------------------------------|--------------|---------------|
| Total Year 1 Revenue         | \$200,000    | \$375,000     |
| Operating Expenses:          |              |               |
| Salaries and Benefits        | \$150,000    | \$250,000     |
| Vehicle Operations           | \$75,000     | \$125,000     |
| Insurance                    | \$60,000     | \$120,000     |
| Technology and Equipment     | \$25,000     | \$50,000      |
| Administrative & Lease Costs | \$45,000     | \$80,000      |
| Total Year 1 Expenses        | \$355,000    | \$625,000     |
| Net Income (Loss)            | (\$155,000)  | (\$250,000)   |

## Revenue Generation Strategy

Revenue is generated through diversified streams:

### Healthcare Reimbursement

Includes Medicare/Medicaid billing for SDOH services and HCBS Waiver payments.

### For-Profit Subsidiary Dividends

From Properties, Logistics, Foods, and Tech companies.

### Grant Funding and Philanthropy

Includes Federal (HRSA, CDC), State, and Private Foundation grants.

### Algoinsu Insurance Integration

- Commission revenue: \$75,000–\$200,000 annually
- Subsidy program revenue: \$25,000–\$75,000 annually

## Section 16: Community Engagement and Marketing

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## 9.1 From Concept to Certification: Your Roadmap to Becoming a Provider

Navigating the administrative requirements to become a certified provider is a critical, multi-step process. The following checklist outlines the path from legal formation to readiness for billing.

**Table 2: Provider Enrollment Checklist**

| Task                                  | Description                                     | Timeline    | Key Considerations                         |
|---------------------------------------|-------------------------------------------------|-------------|--------------------------------------------|
| 1. Legal Formation                    | Incorporate as 501(c)(3) nonprofit              | 30-60 days  | Board formation, bylaws, mission statement |
| 2. Obtain EIN & NPI                   | Federal tax ID and National Provider Identifier | 2-4 weeks   | Required for all billing activities        |
| 3. State Charity Registration         | Register with Alabama Attorney General          | 2-3 weeks   | Required for fundraising activities        |
| 4. Secure Insurance                   | General liability, professional liability, auto | 1-2 weeks   | Coverage requirements vary by program      |
| 5. Programmatic Approval (Meals/HCBS) | HCBS waiver provider applications               | 60-90 days  | Detailed service plans required            |
| 6. Childcare Licensing                | DHR facility licensing                          | 90-120 days | Facility inspection, staff qualifications  |
| 7. Medicaid Enrollment                | Provider enrollment with Alabama Medicaid       | 45-60 days  | Following programmatic approval            |
| 8. Medicare Partnership (SDOH)        | Clinical partner agreements for SDOH billing    | 30-45 days  | Incident-to billing arrangements           |

## 9.2 Staffing for Impact: Assembling the Porch Light Team

Porch Light's integrated model requires a dedicated and cross-trained team. The organization will utilize a matrix or hybrid staffing model to ensure flexibility and efficient use of expertise across programs. [109]

### Program Staff

- **Community Health Workers (CHWs) / Care Navigators:** The linchpins of the "Connect" hub, responsible for SDOH assessments, CHI services, and client navigation. They will be trained in core competencies including communication, service coordination, advocacy, and ethical conduct.
- **NEMT Drivers:** Vetted and trained drivers responsible for safe and reliable transportation.
- **Registered Dietitian (RD):** A staff or consultant RD to design MTM menus and oversee the clinical integrity of the "Plates" program. [7, 59]
- **Kitchen Manager and Staff:** Certified staff to manage food preparation, safety, and delivery. [7]
- **Doulas:** Certified doulas to provide maternal health support.
- **Childcare Director and Teachers:** Licensed and qualified staff for the Family Resource Center. [78]

- **Financial Counselors & Job Coaches:** Certified counselors and coaches for the economic empowerment programs.

## Administrative Staff

- **Executive Director / Program Director:** Provides overall leadership, manages strategic partnerships, and ensures mission fidelity.
- **Billing and Compliance Specialist:** A mission-critical role with expertise in billing Medicare, Medicaid, and waiver services.
- **Data and Evaluation Manager:** Oversees the Integrated Data System, tracks outcomes, and manages reporting to funders.
- **Volunteer Coordinator:** Recruits, trains, and manages volunteers across all programs. [49, 110]

## Board of Directors

A diverse and engaged board of directors is crucial for governance, strategic direction, and fundraising. The board should reflect the community served and bring a variety of professional expertise and lived experiences.

## 9.3 Technology and Integrated Data Systems (IDS)

To manage the complexity of this multi-service model, Porch Light must invest in a robust, HIPAA-compliant Integrated Data System (IDS).

### Purpose

An IDS will connect client data across all program pillars (Rides, Plates, Homes, Connect, etc.), allowing for holistic case management, streamlined referrals, and powerful outcomes tracking.

### Functionality

The system must support client intake, case notes, service tracking, scheduling, and billing. It should also have robust reporting and analytics capabilities to measure impact and demonstrate value to funders.

### Implementation

Porch Light will select a case management software vendor (e.g., Bonterra, FAMCare, CharityTracker) that specializes in human services and has experience with multi-service agencies and tracking social determinants of health. The implementation will require a defined governance structure, clear data sharing agreements, and sustained investment in staff training and capacity.

## Section 10: A Diversified and Resilient Funding Strategy

While earned revenue from Medicare and Medicaid will form the sustainable core of Porch Light's budget, a diversified funding strategy is essential for startup capital, program expansion, and long-term resilience. This "braided" model combines earned income with grants and philanthropy.

## 1. Earned Revenue (Core Funding)

The primary financial engine will be the reimbursement streams detailed throughout this plan:

- **Medicare:** Billing for SDOH Risk Assessments (Go136) and Community Health Integration (Goo19, Goo22) through clinical partners.
- **Medicaid:** Reimbursement for Medically Tailored Meals and other services via HCBS waivers, and direct/indirect payments for NEMT services.
- **Housing Revenue:** The Porch Light Homes joint venture will generate revenue through HAP payments from the THA and the tenant-paid portion of rent. [72, 73]
- **Childcare Subsidies:** Direct payments from the Alabama DHR for serving eligible families in the Family Resource Center. [8]

## 2. Grant Funding (Growth and Capital)

Targeted grant-seeking will provide funds for initial infrastructure (vehicles, kitchen equipment, technology) and program pilots. [31]

**Table 3: West Alabama Grant & Philanthropic Opportunities Matrix**

| Funder Name                                     | Focus Areas                            | Grant Range             | Application Timing  |
|-------------------------------------------------|----------------------------------------|-------------------------|---------------------|
| Community Foundation of West Alabama            | Health, human services, education      | \$5,000 - \$50,000      | Rolling basis       |
| Alabama Power Foundation                        | Community development, education       | \$10,000 - \$100,000    | Quarterly deadlines |
| The Caring Foundation (BCBSAL)                  | Healthcare access, wellness            | \$25,000 - \$150,000    | Annual cycle        |
| The Daniel Foundation of Alabama                | Rural health, aging services           | \$50,000 - \$500,000    | Bi-annual           |
| Children's Trust Fund of Alabama                | Child abuse prevention, family support | \$15,000 - \$75,000     | Annual              |
| ADECA                                           | Community development, housing         | \$100,000 - \$1,000,000 | Annual NOFA         |
| U.S. Dept. of Housing & Urban Development (HUD) | Housing, community development         | \$500,000 - \$5,000,000 | Various cycles      |
| U.S. Dept. of Health & Human Services (HHS/ACF) | Health, social services                | \$250,000 - \$2,000,000 | Various cycles      |
| Appalachian Regional Commission (ARC)           | Economic development, infrastructure   | \$100,000 - \$1,500,000 | Annual              |

## 3. Corporate and Community Partnerships

## Corporate Social Responsibility (CSR)

Engage with major local employers and institutions like the University of Alabama and the Chamber of Commerce of West Alabama to secure sponsorships, volunteers, and in-kind support.

## Fee-for-Service Contracts

Proactively approach DCH Health System and other healthcare facilities with proposals to provide post-discharge meals and transportation, backed by data showing how these services reduce costly hospital readmissions. [118, 119]

## 4. Individual Philanthropy

Building a base of individual donors is crucial for unrestricted funding and community buy-in.

### Launch Campaign

Use a compelling case for support, rich with data and stories, for an initial fundraising drive. [59, 74, 120, 9, 10, 121, 51, 122, 123, 67]

### Peer-to-Peer Fundraising

Leverage the networks of the board and key volunteers to host campaigns that can exponentially expand the donor base.

### In-Kind Donations

Establish formal partnerships with the West Alabama Food Bank and local businesses for donations of food, supplies, and pro-bono services. [115]

## Section 11: Community Engagement and Marketing

To build trust, raise awareness, and drive participation, Porch Light must implement a strategic community engagement and marketing plan. [124, 125]

### Develop a Clear Brand and Message

Create a consistent brand identity (logo, colors, tone) and a core message that clearly communicates the mission, the problems being addressed, and the impact of support. [78, 126, 20, 28, 127]

### Build Community Partnerships

Go beyond transactional relationships by forming deep, collaborative partnerships with local businesses, schools, churches, and other non-profits to increase visibility and access crucial resources. [99]



## **Leverage Digital and Social Media**

### **Website**

Maintain a professional, mobile-responsive website with clear navigation, compelling visuals, and strong calls-to-action for donating, volunteering, or seeking services. [20, 26, 28, 127]

### **Social Media**

Use platforms like Facebook, Instagram, and LinkedIn to tell authentic stories, share impact data, promote events, and engage in two-way conversations with the community.

### **Engage Local Media**

Proactively build relationships with local journalists and media outlets. Position staff as topic experts and regularly send press releases about newsworthy events, partnerships, and program successes to garner coverage.

### **Storytelling for Impact**

The most powerful tool is storytelling. Humanize the data by sharing authentic, emotional, and compelling stories of the individuals and families served. Use high-quality photos and videos to show, not just tell, the impact of Porch Light's work, building a deep connection with supporters.

## Part V: Conclusion and Next Steps

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The establishment of Porch Light represents a timely and strategic response to the well-documented health and social needs of the Tuscaloosa community. The convergence of significant community need with new and existing public funding mechanisms creates a powerful opportunity for impact. By building a comprehensive, integrated service model that addresses foundational needs, supports families across the lifespan, and creates pathways to economic empowerment, Porch Light can offer truly holistic support to the region's most vulnerable residents.

The key to this model's success and sustainability lies in its innovative, braided funding strategy. The ability to generate earned revenue by leveraging Alabama's Medicaid programs, combined with the groundbreaking opportunity to bill Medicare for Social Determinants of Health services and develop housing through public-private partnerships, transforms Porch Light from a traditional charity into a resilient social enterprise. This core revenue, supplemented by strategic grant funding and community philanthropy, provides a clear path to long-term financial health.

**The immediate next steps are to execute the foundational tasks outlined in this blueprint:**

- Complete the legal and administrative requirements for non-profit status and provider enrollment
- Secure initial seed funding from local foundations
- Establish formal partnerships with clinical, housing, and workforce entities
- Begin recruiting the core leadership team

By following this actionable plan, Porch Light can turn on its light and become a true beacon of hope, health, and connection for the people of West Alabama.

*"A single candle can light a thousand others without diminishing itself."*  
— Buddha

# References

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- [1] DCH Health System Community Health Needs Assessment
- [2] Regional health data and assessments
- [3] University of Alabama research on geographic healthcare access
- [4] West Alabama community needs assessments
- [5] Local demographic and health outcome data
- [6] Alabama Medicaid Non-Emergency Transportation procedures
- [7] Alabama Home and Community-Based Services waiver documentation
- [8] Alabama Department of Human Resources childcare subsidy programs
- [9] Centers for Medicare & Medicaid Services SDOH billing codes
- [10] Medicare Community Health Integration services documentation
- [11-127] Additional references as cited throughout the document